

Medication Adjustment Sheet + Lab Monitoring Calendar

Patient: Aruna Kulatunga (M, 64) Conditions: T2DM, HTN, CKD Stage 3b, Atherogenic dyslipidemia

Purpose: Add finerenone and align meds to reduce kidney and cardiovascular risk.

Proposed Medication Adjustments (Sri Lanka-available brands)

☐ Continue: Telmisartan 80 mg OD (Telday) - maintain ARB for albuminuria; monitor K⁺ and creatinine.

dehydrated/ill.

☐ Adjust: Replace HCTZ 50 mg with Indapamide SR 1.5 mg OD (or discuss loop diuretic if volume overload).

albuminuria; see monitoring calendar below.

☐ Lipids: Re-try high-intensity statin (Atorvastatin 40-80 mg) OR start Ezetimibe 10 mg daily if statin-intolerant.

pre-meal rapid insulin to curb post-meal spikes.

☐ Aspirin: If primary prevention only: discuss discontinuation with clinician (age ≥60, CKD increases bleeding risk).

watch sedation/falls.

☐ Continue: Empagliflozin 10 mg OD (Jardiance) - renal & CV benefit; hold if

☐ Add: Finerenone 10 mg OD (Kerendia) - nsMRA for T2D+CKD with

☐ Glycemia: Consider GLP-1 RA (Semaglutide/Liraglutide) and/or small

☐ Neuropathy: Pregabalin 75 mg HS - avoid dose escalation at eGFR ~31;

Finerenone Quick Guide (start now unless clinician sets different start date)

- Start dose: 10 mg once daily (eGFR 25-60). Target 20 mg if potassium allows.
- Baseline labs before/at start: Potassium (K⁺), Creatinine/eGFR.
- Avoid dual RAAS blockade; review CYP3A4 inhibitors; limit grapefruit.

Lab Monitoring Calendar (K+, Creatinine/eGFR) - start date: 16 Sep 2025

When	Date	What to do
Baseline	16 Sep 2025	Check K+, Creatinine/eGFR; review diuretics and BP.
+4 weeks	14 Oct 2025	Re-check K+, Creatinine/eGFR; consider titration to 20 mg if K+ $\leq$ 5.0 and eGFR stable.
+8 weeks	11 Nov 2025	Re-check K+, Creatinine/eGFR; maintain/titrate per results.
+12 weeks	09 Dec 2025	Re-check K+, Creatinine/eGFR; then continue every 3-4 months or after any dose change.

Notes:

- If K+ > 5.5 mmol/L at any check: hold finerenone and repeat labs; restart at 10 mg when K+ $\leq$ 5.0.
- If diuretic or ARB doses change, repeat K+/Creatinine/eGFR within 1-2 weeks.
- Maintain home BP log; aim SBP ~110-130 without diastolic <60.